



PROCEDURE

Death of A Patient

Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	All NMHS Mental Health (MH) Areas

1. Aim

North Metropolitan Health Service Mental Health (NMHS MH) will practice and respond to the death of a patient in accordance with the [WA Health Review of Death Policy \(MP 0098/18\)](#), the Office of the Chief Psychiatrist's (OCP) [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#), the [Western Australia \(WA\) Coroners Act 1996](#), and the [NMHS Morbidity and Mortality Review Policy](#).

All deaths of involuntary patients are reportable under the [MHA 2014](#) and must be reported to the Coroner.

All cases of unexpected deaths or deaths of unnatural causes of voluntary patients in hospital or the community may be reportable following discussion with the Coroner.

Reviews of patient deaths provide an important opportunity to examine the care afforded to a patient to determine if best-practice treatment was provided and also to identify possible areas for improvement in service delivery.

Patients who are in referral, active/admitted, or have had a referral closed or been discharged from a service within 28 days of the death occurring are within scope for review.

By following this procedure, deaths will be correctly categorised in terms of preventability, and, if identified as a preventable death, this should be notified and investigated as a SAC1 clinical incident.

2. Risk

Culture	☒	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Process

3.1 Inpatients under the MHA 2014 (reportable deaths)

- During normal business hours the Shift Coordinator will contact the Clinical Nurse Specialist (CNS) or Clinical Nurse Manager (CNM) who will inform the Coordinator of Nursing (SRN7) or their delegate. The CNS or CNM or their delegate will inform the patient's medical team during business hours. Support will be provided as required.
- For any deaths occurring outside business hours, the Shift Coordinator will contact the After Hours Nurse Manager who will advise the Duty Medical Officer and Duty Consultant Psychiatrist after hours. The after-hours Nurse Manager will inform the on-call Coordinator of Nursing (SRN7) who will then contact the Executive on-call NMHS MH. Advice and support to be provided as required.
- It is a Medical Officer's responsibility to ensure they are familiar with what constitutes a reportable death as stipulated in Part 1, s.3 of the [Coroners Act 1996](#). Also refer to the Death in Hospital Form ([Appendix 2](#)).
- If a death is anticipated, such as, but not limited to, patients receiving palliative care, the WA Police Coronial Investigation Squad (CIS) may be contacted (08 9267 5700) to confirm under what circumstances the death of that patient would be considered reportable to them. Further contact information for the CIS is available [here](#).
- If the death is clearly not within the criteria of a reportable death, or advice had previously been received from the CIS and is documented in the medical record that the death is not reportable, a further contact is not required.
- If the death is reportable, the WA Police CIS will inform the family first (face to face) of the patient's death.
 - If the Next of Kin (NOK) and /or Personal Support Person (PSP) contacts the Hospital by telephone before the CIS has informed them of the patient's death, Ward staff should put the phone call through to the Team Psychiatrist or Duty Psychiatrist who will promptly determine the most appropriate method and personnel to inform the person.
 - If the NOK / PSP attends the Hospital, the Treating Psychiatrist or Duty Psychiatrist will determine the most suitable method and personnel to inform them.
- Notification of the NOK / PSP by telephone should be avoided wherever possible by the clinical staff.
- The attending Medical Officer must complete a Mental Health Death in Hospital form (PMR 0) as soon as practicable (within maximum period of 48 hours) for all deaths, including those that are not reportable to the CIS. In the case of expected deaths the medical team will inform the deceased's family/ carer. PMR 0 is to be filed in the patients' medical file. The attending Medical Officer will also notify the Head of Clinical Service (HOCS) (or Duty Psychiatrist after hours).
 - During **normal business hours** the treating Psychiatrist will inform the Head of Clinical Service (HOCS) who will notify the Medical Co-director who will advise the NMHS MH Executive Director (ED) in the case of unexpected or high profile / high



risk deaths, who will advise NMHS Chief Executive (CE). For deaths of prisoners and Custody Order (Conditional Order) patients the relevant prison / Mentally Impaired Accused Review Board (MIA) Board will be notified by State Forensic Mental Health Service (SFMHS) HOCS or delegate.

- **After hours or on weekends and public holidays**, the Duty Psychiatrist will inform the NMHS MH Executive on Call who will assess the incident and inform the Executive Director/ NMHS Chief Executive in the case of high profile / high risk deaths. For deaths of prisoners and Custody Order (Conditional Order) patients the relevant prison / Mentally Impaired Accused Review Board (MIA) Board will be notified by Duty Psychiatrist or delegate.
- Where relevant, the CNS or their delegate is responsible for notifying the Chaplaincy or respective cultural representative/nominated person who will ensure an appropriate minister of religion is notified.
- Where a death is anticipated, such as, but not limited to, patients receiving palliative care, the Head of Clinical Service will ensure a mortality review is completed using the [NMHS Mental Health Morbidity and Mortality Case Review Form](#), within 28 days from date of death, or date of the service becoming aware of death in the case of patients discharged from services at the time of death, and in accordance with the NMHS Morbidity and Mortality Review Policy.
- In the case of unexpected deaths, the Shift Coordinator will ensure the incident is reported in DatixCIMS as per [NMHS Clinical Incident Management Policy and Procedure](#).
- The death, inclusive of those that are anticipated or due to natural causes, will be reported as per the [Chief Psychiatrist of Western Australia Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#).

3.2 Care and Movement of the Deceased

- The deceased, with all clothing, equipment and environment is to be left undisturbed. The area is to be secured until directed by the CIS. The CIS is responsible for removal of the body.
- The SRN closes off the medical record after the circumstances surrounding the death have been recorded. The medical record and the documentation listed on the Coronial Inquiry Documentation Checklist ([Appendix 1](#)) is then secured in the Medical Record Department safe until collected and held for safe keeping by the Freedom of Information (FOI) Coordinator.
- The CIS are given a copy of the medical records, as requested while the closed original medical file remains in Medical Records. CIS contact information is available [here](#).
- The CIS may also take statements or ask for further information.
 - Staff who are requested to provide a statement for the CIS are entitled to seek legal advice before provision of a statement and may discuss this matter with the Senior Nurse or HOCS as relevant.
 - If the CIS wishes to take statements from patients, their Treating Team (or after hour medical officer/ Duty psychiatrist) will be consulted regarding suitability / capacity to



be interviewed. Ward staff and duty medical staff will consider appropriate timing and organisation of debriefing/explanation to other patients. The need for ongoing support for the patients' needs will be assessed and provided as required by their treating team in collaboration with their NOK/PSP.

- For deaths deemed to be non-reportable following discussion with the Coroner if prior arrangements for removal of the body of the deceased have not been confirmed, family are to be contacted or make arrangements for this to occur. If reasonable efforts have been made to liaise with family regarding these arrangements, and a decision has not been able to occur within 4 hours, the health service can make alternate arrangements for removal of the body.
- The Multidisciplinary Team (MDT) will:
 - Meet as soon as practicable to discuss the case
 - Consider involving the Aboriginal Hospital Liaison Service where appropriate.
 - Offer to meet the NOK / PSP at the earliest opportunity to discuss the circumstances surrounding the death.
 - If no response is forthcoming from the NOK / PSP within two weeks of the initial offer being made, a follow up letter will be forwarded.
 - Compose a letter of sympathy to the NOK / PSP signed by the Consultant Psychiatrist or another delegated member of the team, and sent by registered mail.
 - Initiate next steps towards a SAC1 clinical incident investigation or mortality review as indicated.

3.3 Community Settings

In the case of all community deaths

- The staff notified of the death will inform the Service Coordinator/Manager immediately who will request the notification in writing from the CIS.
- The Service Coordinator/Manager will then notify the Local Leadership Team and Head of Clinical Service (HOCS) who will notify the Medical Codirector during business hours and NMHS MH Executive on-call after hours.
- Where a death is anticipated, such as, but not limited to, patients receiving palliative care, the Head of Clinical Service will ensure a mortality review is completed using the [NMHS Mental Health Morbidity and Mortality Case Review Form](#), within 28 days from date of death, or date of the service becoming aware of death in the case of patients discharged from services at the time of death, and in accordance with the NMHS Morbidity and Mortality Review Policy.
- In the case of unexpected deaths, the Service Coordinator/Manager will ensure the incident is reported in Datix CIMS as per [NMHS Clinical Incident Management Policy and Procedure](#).
- The Service Coordinator/Manager closes off the medical record after the circumstances surrounding the death have been recorded. The medical record is stored in the clinic. CIS requests for copy of the medical file will be facilitated by the FOI Coordinator.



-
- The CIS may also take statements or ask for further information. Staff who are requested to provide a statement for the CIS are entitled to seek legal advice through Legal Services before provision of a statement and may discuss this matter with the Local Leadership Team or HOCS as relevant.
 - The Multidisciplinary Team (MDT) will:
 - Meet as soon as practicable to discuss the case.
 - Initiate next steps towards a SAC1 clinical incident investigation or m review as indicated.

3.4 Accessing Post Mortem Reports

- Access to Post Mortem Reports is currently on hold for services. This process is being reviewed.

3.5 Employee Assistance and Wellbeing

The [NMHS Critical Incident Process](#) intranet page contains the relevant information to guide managers with incident debriefing, staff check in and ongoing support,

- [Critical Incident – Post incident support](#)
- [Critical Incident Response: Post Incident Debriefing \(checklist\)](#)
- [Critical Incident Debrief Session Information Sheet](#)

All incidents with the potential to cause significant harm to staff should be reported via the [Incident and Hazard Report Form](#). If any staff report concerns about their personal reactions to an event, they should be directed to NMHS Health Safety and Wellbeing for support.

The Co-Directors or Local Leadership Team will provide support to staff involved in the patient death including information regarding [Employee Assistance Program \(EAP\)](#).

NMHS MH has two employee assistance providers:

- a) [Converge International](#)
 - 1300 687 327 (free call)
 - <https://convergeinternational.com.au/>
- b) [PeopleSense](#)
 - 1300 307 912 (free call)
 - <https://www.peoplesense.com.au/psychological-services/clinical-services/eap-counselling>

4. Performance and Monitoring

- 100% of all deaths not already addressed as SAC 1 investigations through the clinical incident management system (CIMS) are to be reviewed using the [NMHS Mental Health Morbidity and Mortality Case Review Form](#), within **28 days from date of death**, or date of the service becoming aware of death in the case of patients discharged from inpatient and

community services at the time of death, and in accordance with the NMHS Morbidity and Mortality Review Policy.

- The following documentation will be completed in 100% of cases of death of an inpatient where the death is unexpected:
 - DATIX CIMS
 - Coronial Inquiry Documentation Checklist ([Appendix 1](#))
 - Death in Hospital form (PMR 0) - This form is also required for 100% of expected deaths.
- The following data is reported to the Patient Safety and Surveillance Unit as per the [WA Health Review of Death Policy \(OD 0098/18\)](#) within **3 months** following the date of death, via the MHPHDS Clinical Risk Management Coordinator:
 - Total number of deaths.
 - Deaths notified as SAC1 incidents.
 - Deaths referred to WAASM (Western Australian Audit of Surgical Mortality).
- Monitoring of this procedure and corresponding data is to be tabled at the NMHS MH Clinical Governance Committee (CGC).

Related internal policies, procedures and guidelines

[NMHS Critical Incident Response: Post Incident Debriefing Checklist](#)
[NMHS Clinical Care of People Who May Be Suicidal Policy](#)
[NMHS Clinical Incident Management Policy and Procedure](#)
[NMHS Morbidity and Mortality Review Policy](#)

References

[WA Health Clinical Care of People Who May Be Suicidal Policy \(MP 0074/17\)](#)
[WA Health Review of Death Policy MP 0098/18](#)
[Mental Health Act 2014](#)
Chief Psychiatrist - [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
[WA Coroner Act 1996](#)

Useful resources

[Datix CIMS](#)
[NMHS MH Morbidity and Mortality Case Review Form](#)
[Coronial Inquiry Documentation Checklist \(\[Appendix 1\]\(#\)\)](#)

[Office of the Chief Psychiatrist](#)
[Coroner's Court of Western Australia – Family Information](#)
[Coroner's Court of Western Australia – Family Information, Practical Issues](#)
[Wellbeing at NMHS \(Workplace Health and Safety\)](#)
[NMHS Critical Incident Process](#)
[MHPHDS Wellbeing Program](#)
[Supporting Staff with Grief: A Guide for Leaders](#)

Policy Sponsor	Co-Director, Adult Inpatient				
Policy Contact	Co-Director, Adult Inpatient				
Date First Issued:	01/04/2007	Last Reviewed:	19/10/2023	Review Date:	19/10/2026
Approved by:	MH Policy Coordination Committee			Date:	19/10/2023
Endorsed by:	Executive Director, MHPHDS			Date:	15/11/2023
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance				
Chief Psychiatrist's Standards for Clinical Care:	☒ Risk Assessment and Management,				
Printed or personally saved electronic copies of this document are considered uncontrolled					

AUTHORISATION: (For MH policies only – delete this box if not applicable)			
Committee Authorising Document	Date Authorised	Minutes Ref. No.	Document Reference (eg MHEG Meeting Minutes)
MHEG	April 2007	19/07	MHEG Minutes
Acting Executive Director	24/4/2015	Signed on previous version	Out of session (MHEG Item 131/15 May 2015)
Area Medical Advisory Committee	09/03/2020		
MHEG	14/06/2018	-	Out of session via email
Executive Director	18/06/2018	-	Submission form
Area Medical Advisory Committee	09/03/2020		
MH Management Committee	12/11/2020		Out of session endorsement via email. See Policy folder in W Drive
Executive Director	19/11/2020		Submission Form
MH Policy Coordination Committee	19/10/2023		18/10/23 Meeting minutes

Document Version Control			
Date	Version	Author	Notes
18/06/2018	2.0	Risk Management Coordinator	Replaces "Review of Deaths" policy (which was previously WA Review of Mortality). Significant review to include more detail for inpatient processes and include community processes not previously included.
01/10/2018	2.1	A/Policy & Audit Officer	Inclusion of Mental Health, NMHS, and DoH Clinical Care of People who may be Suicidal policy documents.
03/10/2018	2.2	A/Policy & Audit Officer	Inclusion of step 'f' in process in the 'Accessing Post Mortem Reports' section.
19/11/2020	3.0	MHPHDS Policy & Audit Officer	Revised advice regarding involvement of CIS in contacting family of deceased. All links updated. Lines of intra-service communication clarified. Update to title of Coronial Investigation Squad.
04/11/2021	4.0	MHPHDS Policy & Audit Officer	Amended the Procedure to include clarification and additional forensic information for reporting death.
22/11/2022	4.1	Director of Clinical Services MHPHDS	Minor amendment - removal of MH Management of Death of an Inpatient in a Mental Health Setting During COVID 19 Pandemic Procedure (rescinded) and update of all hyperlinks.
12/10/2023	4.2	Inpatient Co-Director	Amended to incorporate the mortality and morbidity review process.



Appendix 1 – NMHS MH Coronial Enquiry Documentation Checklist

Document Title	☒
Observation Record Chart (ORC)	
Code Blue/ Medical Emergency (MET) Report PMR0001	
Specials and Clinical Observation forms PMR29	
Current Visual Observation checklist and for previous 24 Hours	
Current iSOFT handover and for previous 24 Hours	
Current Ward report and for previous 24 Hours	
Collated report for previous 24 Hours	
PSOLIS Management Plan	
NMHS MH Admission to Discharge Policy (Inpatient)	
Admission of Patients procedure relevant to area (if applicable)	
NMHS MH Death of a Patient Procedure	
Terminally Ill Consumers procedure relevant to area (if applicable)	
NMHS MH Recognising and Responding to Acute Deterioration Policy (RRAD)	
Clinical Deterioration procedure relevant to clinical area (if applicable)	
NMHS MH Clinical Record Documentation Procedure	
Clinical Documentation procedure relevant to clinical area (if applicable)	
Copy of Ward/Unit Orientation	

Appendix 2: Death in Hospital Form

Affix Hospital Identification Here DEATH IN HOSPITAL	SURNAME		UMRN	
	GIVEN NAMES		DOB	GENDER
	ADDRESS			
	SUBURB		POSTCODE	

SECTION 1: EXTINCTION OF LIFE

Death occurred in: _____ ED ☐ Ward ☐ Theatre ☐ Other ☐ While on leave from hospital ☐

Doctor Certifying Life Extinct: Name: _____ Position: _____

Signature: _____ Date: _____

Date of Death: ____ / ____ / ____ dd/mm/yy Time of Death: ____ : ____ 00:00 hours

SECTION 2: IS THE DEATH REPORTABLE TO THE CORONER?

- Is the cause of death unknown or uncertified by a medical practitioner? _____ ☐ Yes ☐ No
- Has the death or does the death appear to be have occurred in suspicious circumstances?
i.e. Has the death possibly resulted from a criminal act? _____ ☐ Yes ☐ No
- Was the death or does the death appear to have been unexpected or unnatural?
e.g. Complication following administration of a medication, diagnostic, medical or surgical procedure _____ ☐ Yes ☐ No
- Has the death or does the death appear to have occurred, in or following violent circumstances?
e.g. Physical or sexual assault, domestic dispute _____ ☐ Yes ☐ No
- Has the death or does the death appear to have resulted, directly or indirectly from injury?
e.g. Fall, motor vehicle, self harm _____ ☐ Yes ☐ No
- Has the death occurred during anaesthesia? e.g. General anaesthesia _____ ☐ Yes ☐ No
- Did the death possibly occur as a result of, or does it appear to have resulted from, anaesthesia? _____ ☐ Yes ☐ No
- Immediately prior to the death was the deceased a person:
 - Under the control, care or custody of the WA Police Force, Prison Service or Child Protection and Family Support _____ ☐ Yes ☐ No
 - Admitted to a centre under the *Alcohol and Other Drugs Act 1974* _____ ☐ Yes ☐ No
 - An involuntary patient, apprehended, detained or absent without leave under the *Mental Health Act 2014* _____ ☐ Yes ☐ No
- Is the deceased person's identity unknown? _____ ☐ Yes ☐ No
- To your knowledge has any one expressed any concerns regarding the cause of the deceased person's death or medical treatment? _____ ☐ Yes ☐ No

If you have answered YES to any of the above questions, the death is REPORTABLE to the Coroner, and the most recent medical records leading up to the person's death must be provided to the police immediately or, in any case, to the State Mortuary not more than 24 hours following death. This is to ensure that the post mortem examination can be conducted in a timely manner.

Where records are paper-based, the originals are to be provided with copies retained by the health service for any ongoing purposes. Should further medical information be requested by the police/coroner these records should then be provided as soon as practicable.

SECTION 3: HOW TO REPORT A DEATH TO THE CORONER

To report a death to the Coroner, or to seek guidance about reportable deaths:

METROPOLITAN PERTH:
Contact the WA Police Coronial Investigation Squad (CIS) on 08 9267 5700 (24 hours-a-day, 7 days-a-week).
Scan and email this form to: Coronial.Investigation.Squad@police.wa.gov.au

COUNTRY WA SETTINGS:
Contact the local police. If further guidance or assistance is required, contact the CIS on 08 9267 5700. The Office of the State Coroner (OSC) can be contacted on 08 9425 2900 (business hours) or on 0419 904 478 (after 4:30pm and on weekends) if the CIS and local Police are unable to assist.

DEATH IN HOSPITAL

MD Y Y Y

Affix Hospital Identification Here DEATH IN HOSPITAL	SURNAME		UMRN	
	GIVEN NAMES		DOB	GENDER
	ADDRESS			
	SUBURB		POSTCODE	

SECTION 4: NOTIFICATION OF REPORTABLE DEATHS UNDER THE CORONERS ACT 1996

On notification you will be asked for information on the circumstances surrounding the death, which should be conveyed to the coronial delegate (e.g. the CIS/police officer to whom the death is reported). You should also consider the answer to the question **"Was death an inevitable consequence of the deceased person's primary illness or condition regardless of appropriate resuscitation, anaesthesia, or surgery?"** In cases when the answer is 'yes' a post mortem may not be necessary.

CIS/police officer notified by phone: Date: ____/____/____ dd/mm/yy Time: ____:____:00 hours

Name of CIS/police officer: (please print) _____

Name of Doctor reporting: (please print) _____

Consultant notified: Yes ☐ No ☐ Contact Number: _____ Sign: _____

If the death is reportable a copy of this Form should also be placed in the deceased person's Hospital Medical Record as well as the Hospital's Coronal Investigation File.

Where the death is NOT reportable:

- the original copy of this Form must be filed in the deceased person's Hospital Medical Record
- you may complete the following Forms:

1. Medical Certificate Cause of Death (BDM 202/201) and	Completed?	☐ Yes ☐ No
2. Certificate of Medical Attendant (Form 7 WA Cremation Act)	Completed?	☐ Yes ☐ No

SECTION 5: OTHER REPORTING OBLIGATIONS

5.1 REPORTABLE DEATHS UNDER THE HEALTH (MISCELLANEOUS PROVISIONS) ACT 1911

Is the death:

- a maternal death (arising from pregnancy or childbirth or associated with complications) ☐ Yes ☐ No
- one involving a child who is stillborn (> than 20 weeks gestation), or under the age of 1 year ☐ Yes ☐ No
- one that occurred within 48 hours of administration of anaesthetic or as a result of complications arising from the same ☐ Yes ☐ No

If you have answered YES to any of the above questions, the death is reportable to the Chief Health Officer. Information on reporting processes is found on the WA Department of Health Statutory medical notifications website: http://ww2.health.wa.gov.au/Articles/A_E/About-statutory-medical-notifications-in-Western-Australia

5.2 DEATHS REPORTABLE TO THE CHIEF PSYCHIATRIST

The Chief Psychiatrist is to be informed of any death of a mental health patient while under the care of any mental health or other health service, and deaths that occurred within 28 days of discharge or deactivation from mental health services. Is this a reportable death to the Chief Psychiatrist? ☐ Yes ☐ No

For further information refer to the Chief Psychiatrist website: <https://www.chiefpsychiatrist.wa.gov.au/monitoring-reporting/notifiable-incidents/>

5.3 SEVERITY ASSESSMENT CODE (SAC) 1 CLINICAL INCIDENTS

The death may reflect the occurrence of a SAC 1 clinical incident. SAC 1 clinical incidents include all clinical incidents/ near misses where serious harm or death is or could be specifically caused by healthcare rather than the patients underlying condition or illness. For further information refer to the Clinical Incident Management (CIM) Policy: <http://ww2.health.wa.gov.au/Health-for/Health-professionals/Safety-and-quality>

SECTION 6: ADDITIONAL HOSPITAL REQUIREMENTS (as determined by local Policies /Guidelines)

Donor Coordinator Notified	☐ Yes ☐ No	Discharge summary completed	☐ Yes ☐ No
Permission for postmortem	☐ Yes ☐ No	Bereavement support	☐ Yes ☐ No
Nursing home notified	☐ Yes ☐ No	General Practitioner notified	☐ Yes ☐ No
Next of kin notified as designated in the Hospital Medical Record ☐ Yes ☐ No			

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (IS21P0118).

This document can be made available in alternative formats on request for a person with a disability.

© North Metropolitan Health Service 2021

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.